PATIENT CHART EXTRACT - VICTORIA GENERAL HOSPITAL (TEST)

MADE-UP TEST PATIENT - NOT A REAL PERSON - FOR SANDBOX TESTING ONLY

Patient: TESTPATIENT, Gwen DOB: 1960-05-21 Sex: F
MRN: TEST60417 Location: 6A General Surgery, Bed 12
Admitted: 2026-11-04 (General Surgery) Attending: Dr. A. Surgeon (made-up)

GENERAL SURGERY ADMISSION NOTE

Date of service: 2026-11-04 09:40
Chief complaint: Abdominal pain, vomiting and no flatus for 2 days.

HISTORY OF PRESENT ILLNESS

66-year-old woman with 3 days of intermittent crampy central abdominal pain, bilious vomiting x 6, no bowel movement or flatus for 48 h. Two weeks of intermittent right upper quadrant discomfort after meals that settled on its own. No fever at home. No prior bowel obstruction. Only prior abdominal surgery: laparoscopic tubal ligation (1992). Never had a cholecystectomy. No history of ERCP or sphincterotomy.

PAST MEDICAL HISTORY

COPD (GOLD 3, home O2 not required, baseline SpO2 92%)
Morbid obesity (BMI 47)
Hypertension
Obstructive sleep apnea, CPAP at night

HOME MEDICATIONS

Tiotropium 18 mcg inhaled daily
Salbutamol 100 mcg inhaled q4h prn
Acetaminophen 500 mg PO prn
Amlodipine 5 mg PO daily (held on admission, NPO)

ALLERGIES: Codeine (nausea)

EXAMINATION

2026-11-04 08:50 BP: 148/84 HR: 96 RR: 20 Temp: 37.2 C SpO2: 92% Weight: 128 kg
Distended, tympanic abdomen, diffusely tender, worst in RLQ. No peritonism. Healed laparoscopic port scars. No hernias.

ASSESSMENT AND PLAN

Small bowel obstruction, presumed adhesive (prior tubal ligation).
NPO, NG tube to drainage, IV fluids, analgesia.
Water-soluble contrast challenge after NG decompression.
CT reviewed: pneumobilia noted, likely incidental.

LABORATORY RESULTS

Collected: Nov 4, 2026 06:30

Test	Result	Units	Reference	Flag
Hemoglobin	138	g/L	120 - 160	
White Blood Cell Count	11.2	x10^9/L	4.0 - 11.0	H
Platelets	286	x10^9/L	150 - 400	
Sodium	138	mmol/L	135 - 145	
Potassium	3.6	mmol/L	3.5 - 5.0	
Creatinine	78	umol/L	45 - 90	
Glucose	7.8	mmol/L	3.6 - 7.8	
Lactate	1.4	mmol/L	0.5 - 2.0	
Lipase	42	U/L	10 - 60	
Total Bilirubin	14	umol/L	0 - 20	
Alkaline Phosphatase	168	U/L	40 - 120	H
ALT	48	U/L	0 - 35	H
INR	1.1	ratio	0.8 - 1.2	

GENERAL SURGERY PROGRESS NOTE - HD2

Date of service: 2026-11-05 08:15
NG output 1.4 L overnight. Pain unchanged. No flatus.
2026-11-05 07:30 BP: 142/80 HR: 98 RR: 20 Temp: 37.4 C SpO2: 91%
Water-soluble contrast challenge #1 given yesterday 18:00.
24 h abdominal X-ray: contrast in dilated small bowel, none in colon. Failed.
Plan: continue conservative management, repeat contrast challenge tomorrow.

GENERAL SURGERY PROGRESS NOTE - HD3

Date of service: 2026-11-06 08:05
Still no flatus. NG output 1.1 L. Mild increase in abdominal pain.
2026-11-06 07:45 BP: 138/76 HR: 104 RR: 22 Temp: 37.6 C SpO2: 90%
Water-soluble contrast challenge #2: 24 h film shows no contrast in colon. Failed.
Plan: continue NG and IV fluids, reassess for OR if no progress in 24 h.

LABORATORY RESULTS

Collected: Nov 6, 2026 06:20

Test	Result	Units	Reference	Flag
Hemoglobin	134	g/L	120 - 160	
White Blood Cell Count	12.6	x10^9/L	4.0 - 11.0	H
Sodium	136	mmol/L	135 - 145	
Potassium	3.4	mmol/L	3.5 - 5.0	L
Creatinine	92	umol/L	45 - 90	H
Magnesium	0.68	mmol/L	0.70 - 1.00	L
Lactate	1.7	mmol/L	0.5 - 2.0	

NURSING NOTE - HD4

Date of service: 2026-11-07 02:40
Patient woke with palpitations and shortness of breath. Monitor shows irregular rhythm, rate 140s-150s. On-call surgery resident notified.
2026-11-07 02:35 BP: 104/62 HR: 152 RR: 26 Temp: 37.9 C SpO2: 88%
O2 increased to 4 L nasal prongs, SpO2 92%.

ECG

Date of exam: 2026-11-07 02:50
Atrial fibrillation with rapid ventricular response, rate 148. Normal axis.
No acute ST elevation. QTc 468 ms. No prior ECG on file.

GENERAL SURGERY ON-CALL NOTE - HD4

Date of service: 2026-11-07 03:20
New atrial fibrillation with RVR, likely secondary to physiological stress of SBO. Digoxin 0.25 mg IV given x 2 doses (03:00 and 07:00). Medicine/hospitalist consult requested in the morning for AF management.

LABORATORY RESULTS

Collected: Nov 7, 2026 05:10

Test	Result	Units	Reference	Flag
Hemoglobin	129	g/L	120 - 160	
White Blood Cell Count	14.8	x10^9/L	4.0 - 11.0	H
Platelets	241	x10^9/L	150 - 400	
Sodium	134	mmol/L	135 - 145	L
Potassium	3.1	mmol/L	3.5 - 5.0	L
Creatinine	124	umol/L	45 - 90	H
Magnesium	0.62	mmol/L	0.70 - 1.00	L
Glucose	9.1	mmol/L	3.6 - 7.8	H
Lactate	2.4	mmol/L	0.5 - 2.0	H
Troponin	41	ng/L	0 - 14	H
TSH	1.9	mIU/L	0.3 - 4.0	
Total Bilirubin	22	umol/L	0 - 20	H
Alkaline Phosphatase	182	U/L	40 - 120	H
ALT	52	U/L	0 - 35	H
INR	1.2	ratio	0.8 - 1.2	

ACTIVE MEDICATION ORDERS (2026-11-07)

Digoxin 0.25 mg IV q4h x 2 doses (completed 07:00)
Enoxaparin 40 mg SC daily
Hydromorphone 0.5 mg IV q3h prn pain
Ondansetron 4 mg IV q8h prn nausea
Pantoprazole 40 mg IV daily
Tiotropium 18 mcg inhaled daily
Salbutamol 100 mcg inhaled q4h prn
KCl 20 mEq IV x 2 runs
Plasma-Lyte 125 mL/h IV

ACTIVE PROBLEM LIST

Small bowel obstruction, presumed adhesive
Atrial fibrillation, new onset HD4
COPD
Hypertension
Obstructive sleep apnea
Morbid obesity

CONSULT REQUEST - MEDICINE / HOSPITALIST

Date of service: 2026-11-07 08:00
From: General Surgery. Reason for consult: new atrial fibrillation with RVR on HD4, presumed due to stress of SBO. Please assess and manage AF.

HOSPITALIST CONSULT NOTE (DRAFT, INCOMPLETE)

Date of service: 2026-11-07 10:30
Consulted by General Surgery for new AF with RVR.
2026-11-07 10:15 BP: 112/68 HR: 128 RR: 24 Temp: 38.1 C SpO2: 91%
Pneumobilia on CT not explained: no ERCP, no sphincterotomy, no biliary surgery.
Gallbladder reported absent but patient has never had a cholecystectomy.
An AI clinical decision tool suggested gallstone ileus (Rigler triad: SBO, pneumobilia, ectopic gallstone), a diagnosis new to me.
Possible gallstone ileus?
To discuss with General Surgery and Radiology.

RADIOLOGY REPORTS

CT ABDOMEN AND PELVIS WITH CONTRAST

Date of exam: 2026-11-04 07:15

FINDINGS:

Multiple dilated fluid-filled small bowel loops up to 3.6 cm. Transition point in the right lower quadrant; distal ileum and colon decompressed. No free air. No pneumatosis.
Small volume free fluid in the pelvis.
Air in the central intrahepatic bile ducts and common bile duct (pneumobilia).
Gallbladder not visualized, presumably surgically absent.
Subtle wall thickening of the first part of the duodenum adjacent to the gallbladder fossa.
Ill-defined 2.4 cm heterogeneous intraluminal focus at the transition point with a faint peripheral rim, favoured to be fecalized small bowel content.

IMPRESSION:

1. Small bowel obstruction with RLQ transition point, likely adhesive.
2. Pneumobilia, likely incidental. Clinical correlation for prior sphincterotomy.
3. Gallbladder not visualized, presumed post-cholecystectomy.